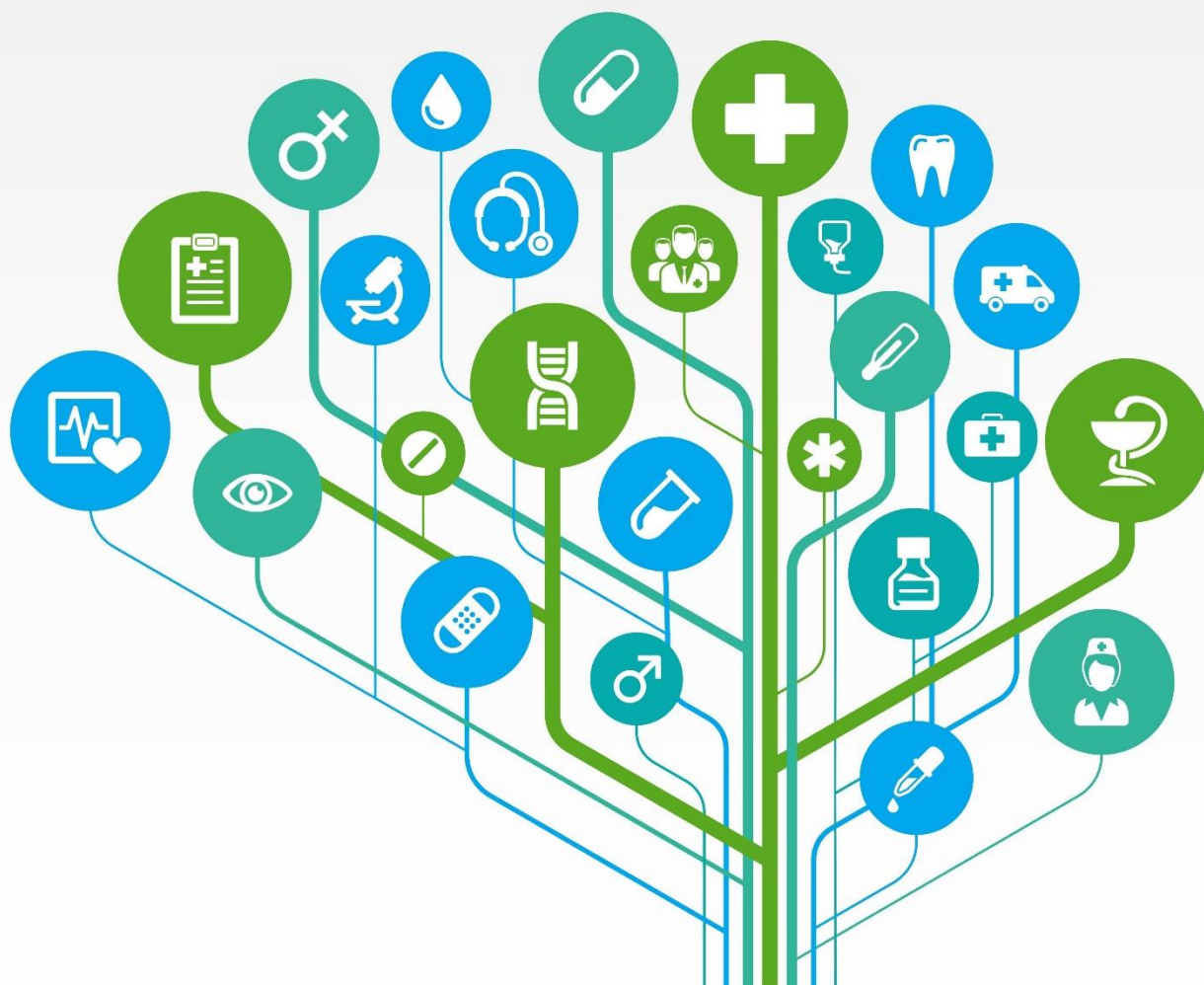




Opioid Use Disorder Treatment

A Toolkit for Oregon Emergency Departments

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Introduction

Background

Substance Use Disorder (SUD), including Opioid Use Disorder (OUD), is a highly prevalent and increasing crisis in Oregon. Sadly, Oregon ranks 1st in the nation for percent of population with illicit drug use disorder as well as 1st in the nation for percent of population with prescription opioid misuse in the last year.¹ While prescription drug overdoses and risky opioid prescribing have seen a recent steady decline in the state, hospitalizations and deaths due to heroin and fentanyl are on the rise² and we are not providing the level of treatment that our patients need to recover. Oregon ranks 50th (worst) in the country for people needing but not receiving treatment for substance use disorder.¹

As a health care community, all members of the care continuum have a responsibility and a part to play in changing the trajectory of this situation and providing compassionate, appropriate care to individuals that need our support.

The clinicians and teams within Oregon’s emergency departments (EDs) have a unique opportunity to intervene during a crucial moment of a patient’s life; to not just stabilize and discharge, but to provide the medications and connections people need to start down a new and healthier path.

The goal of this toolkit is to provide a basic framework for EDs that want to build or strengthen their OUD treatment programs and to offer Oregon-specific resources and examples that can help support their success. In addition, several excellent documents already exist that offer more detailed guidance. These are recommended for all EDs that want to improve their OUD treatment protocols:

- [CA Bridge – Blueprint for Hospital Opioid Use Disorder Treatment](#)
- [SAMHSA – Use of Medication Assisted Treatment in the Emergency Department](#)
- [The National Council on Mental Wellbeing – Addressing Opioid Use Disorder in EDs](#)
- [ACEP Emergency Quality Network Opioid Toolkit](#)

This Toolkit was developed by members of the Oregon Health Leadership Council’s (OHLC) Substance Use Disorder Workgroup, with the guidance and support of OHLC’s [Best Practice Committee](#). These groups are comprised of providers, clinical leaders, and SUD experts representing health systems, clinics, the Oregon Health Authority, and health plans throughout Oregon.

We recognize that identification, treatment, and follow-up for patients with OUD that present to the ED can be time consuming and complicated by co-morbidities, mental health, social needs, available hospital and community resources, and other factors. We also know that the increased use of synthetic opioids like fentanyl add further complexity. As such, OUD treatment processes will vary between regions, hospitals, and patients, and will need to continue to expand and evolve over time. **While it may be challenging at first, current evidence and models show us that it is possible, and that we can do more to systematically and effectively treat patients with OUD in our emergency departments.**

Rationale

There is strong evidence for the effectiveness of ED-initiated medication for opioid use disorder (MOUD) in addition to a secured plan for follow-up services. Patients receiving buprenorphine for OUD are at lower risk of overdose³ and those who are started on buprenorphine in the ED show “significantly increased engagement in addiction treatment, reduced self-reported illicit opioid use, and decreased use of inpatient addiction treatment services”.⁴ With a focused approach using evidence-based practices, adequate resources, and leadership support, improved OUD treatment is achievable in small community hospitals or large academic settings, in rural and urban areas.^{5,6,7} In addition to improved patient outcomes, ED-initiated buprenorphine is of high value, as it is most likely to be cost effective compared to brief intervention or referral to community-based treatment alone.⁸

EDs are an essential place of service to initiate MOUD and facilitate appropriate treatment referrals for patients struggling with opioid use disorder.

Following an opioid overdose, patients who survive are at high risk of death from several causes⁹ within the first 2 days after ED discharge.^{10,11} Buprenorphine has been associated with reduced all-cause and opioid-related mortality, but patients often do not receive the treatment they need^{12,13}. Since many patients do not have established relationships with other providers, ED participation in this service fills a significant gap in the current system of care and allows the only access point that many patients may have to receive treatment.

“Initiation of treatment in the ED can be lifesaving. Even one or two days of medication can prevent an overdose”

*- Josh Leibovitz, MD
Emergency & Addiction Medicine Specialist*

Many Oregon EDs do not currently have adequate processes in place to effectively care for patients with OUD. A [2022 OHLC survey of 24 Oregon hospitals](#) found that:

- Only 21% of EDs have a protocol for initiating buprenorphine; and
- Over 40% of EDs do not have an adequate referral system to ensure appropriate follow-up care

In an analysis of Oregon Medicaid members in the Portland Metro area from July 2019- June 2020, over 9000 people were given an OUD diagnosis. About **18% of these people were diagnosed with OUD for the first time in the ED** and subsequently had a lower rate of treatment than those diagnosed in primary care or outpatient behavioral health.¹⁴

Place of first OUD diagnosis	Final treatment status during the time period		
	Medication supported treatment	Treatment without medication support	No treatment
Emergency Department	25.3%	7.1%	67.6% (1,158 people)
Primary Care	55.2%	2.4%	42.5% (994 people)
Outpatient Behavioral Health	86.9%	11.3%	1.9% (83 people)

Given increasing opioid overdose rates, the emergence of illicitly manufactured fentanyl, and the impact of the COVID-19 pandemic on mental health and substance use, it is crucial that we act now to offer better solutions through Oregon’s EDs. In this toolkit you will find information and resources you will need to start building the relationships and workflows to support and sustain your efforts.

Developing a Successful Program

Address the barriers

Providers generally see the value in initiating opioid treatment in the ED. Most see the inadequacies with the traditional approach of stabilizing immediate needs and discharging patients with a phone number or pamphlet, and they recognize the need for a more effective process. However, ED clinicians do not always feel comfortable prescribing medications for OUD, and the process does produce some operational challenges that must be managed.^{15,16,17}

Logistical barriers commonly include the need for: • Training on all aspects of OUD treatment • Available referral partners and follow-up treatment options • Clear protocols & practice guidelines • Time to educate and discuss with the patient • Support staff to assist with care coordination • Ability to determine patient outcomes • Access to social or financial support for the patient	Resources that help overcome barriers include: • Clear leadership support & involvement • Enthusiastic provider & staff champions • Department-wide trainings & education • Referral agreements & pathways • Efficient electronic workflows & communication • Hiring of appropriate support staff • Available expert consultation • Outcome feedback mechanisms • Continual quality improvement
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While the barriers may seem daunting, they are not insurmountable, and effective models exist across the country. The **resources above are key elements of an OUD treatment program**, described in more detail throughout this Toolkit, that will help facilitate success. In addition to logistical barriers, certain perceptions or beliefs can also contribute to hesitancy to prescribe MOUD. However, consideration of the information below may help relieve some of these concerns.

Perception barriers	Considerations
“OUD is a chronic condition and initiating treatment is not the role of the ED”	• SAMHSA, ACEP, and AAEM all support initiation of buprenorphine as first line treatment for opioid withdrawal in the ED^{18,19,20}
“Buprenorphine will be misused or sold illicitly”	• Many commonly prescribed medications have misuse potential • Misuse is not common – buprenorphine is a partial agonist with less abuse potential than a full agonist • Even 1-3 days of medication from the ED can prevent overdose until outpatient care is accessed
“There will be a large increase in patients coming to the ED for OUD treatment”	• With the elimination of the X-waiver, any DEA licensed provider can prescribe buprenorphine • Many of these patients are already being seen regularly in the ED, treatment will reduce their need for emergency care

Build foundational elements

Prior to implementation of an opioid treatment program in the ED, there are four recommended foundational elements that organizations should develop that will help promote success. These include leadership involvement, ED champions, proper training for the whole team, and established referral processes with community treatment partners.

1. LEADERSHIP INVOLVEMENT

Building and sustaining OUD treatment pathways in the ED requires a team-based approach including providers, support staff, technology infrastructure, and community partnerships. Higher level messaging and supports are necessary to enable and coordinate improvement efforts. Therefore, **commitment by leadership from both the department and organization is the most critical foundational element to success.**

Leadership can support OUD treatment in the ED through the following actions^{15,17,21,22,23,24}:

PROMOTE A CULTURE



- Communicate to the ED team that **OUD treatment is a priority** and that there is a **duty and moral obligation** to offer it to patients
- Uphold the position that **OUD treatment is a skill that belongs within emergency medicine** and is necessary to meet the needs of patients who may not seek care elsewhere
- Provide instruction on **trauma-informed care and harm reduction**

DEDICATE TIME AND RESOURCES



- Arrange time and space for the **collaborative development of policies**, procedures, practice guidelines, and patient materials related to OUD treatment
- Hire and train the **multi-disciplinary staff** needed to support clinicians and facilitate efficient and effective OUD treatment services

FACILITATE EDUCATION



- Arrange **department-wide training** in the areas of addiction medicine, opioid use disorder, pain science, stigma reduction, etc. (see page 7 for education resources)
- **Incentivize providers to obtain continuing education** through offering on-site training, training during shift, or reimbursement for time

2. ED CHAMPIONS

Within each ED, the designation of one or two providers who will champion this effort among their team is essential for the program's success. Nurse champions can also be a valuable addition. Champions will help to facilitate the culture changes, positive attitudes, and quality improvement efforts needed. Champions would ideally volunteer for the role but may be recruited by organization leaders. They should have a special interest in substance use disorder with a desire to promote evidence-based treatment in their ED. Champions can support OUD treatment in the ED through the following actions:^{24,25}

- Ensure the ED team is obtaining adequate education
- Act as a resource during shift to answer questions from prescribers
- Serve as a mentor for staff assisting with screening and referrals for patients with OUD
- Actively participate in the development of program policies, workflows, and clinical guidelines
- Serve as a liaison between the ED and organizational leadership regarding the progress, successes, and needs of the department
- Participate in the hiring and supervision of any clinical navigators or peer support personnel that assist with the management of patients receiving OUD treatment or resources



OREGON Spotlight: ED Champion

"My experience as an SUD champion in the ED has been incredibly rewarding on a personal and professional level. The ED is one of the most common access points that people who use substances have with the health care system. For too long, the status quo has been to treat an overdose or abscess and discharge without further discussing substance use. I have found that my team and I, through the work that I and an RN champion have done, are changing how we interact with patients with SUD. We feel more comfortable having open conversations about a person's use and either starting treatment in the department or having up-to-date treatment information, discussing harm reduction and referrals to peers. **Change can be slow, and there are always bumps along the way, but having a steady team of champions to work through these issues and keep the momentum going, is vital.** I have seen growth and less burnout in myself and my teammates by changing the stigma around SUD and how we communicate with people who use substances. Hopefully, this change will be noticed by the community we serve and rebuild the trust with health care providers."

- *Melissa Willitzer, PA-C*
ED Lead APP: Adventist Health, Hillsboro Medical Center, & Columbia Memorial Hospital

3. EDUCATION

One of the most predominantly cited barriers to addressing OUD in the ED is insufficient training in multiple aspects of the treatment process. Most currently practicing ED providers have not received training in MOUD. However, recently trained providers are more likely to approve of ED-initiated buprenorphine, demonstrating the importance of up-to-date education.¹⁵ It is recommended that EDs offer department-wide trainings for the entire ED team, including providers, nurses, techs, administrative staff, and other support staff.

NOTE: As of January 2023, the **X-waiver** for prescribing buprenorphine, along with the associated training requirements, **was eliminated**. However, new training requirements were implemented for all providers applying for or renewing their DEA license. See page 13 for additional details.

In addition to educating staff on how to treat patients with medication, training should provide a foundational knowledge of addiction medicine, an understanding of laws and regulations pertaining to MOUD, and research evidence regarding the safety and effectiveness of buprenorphine. Organizations should also provide the team with information that will serve to reduce stigma, such as trauma-informed care, harm reduction strategies, brief counseling for patients with OUD, and understanding OUD as a disease.

Education Resources

OREGON EDUCATION RESOURCES	DESCRIPTION
Oregon ECHO Network: Substance Use Disorders in Hospital Care	A 12-session virtual series that addresses clinical care, patients and provider experience, and stigma towards people who use drugs
Opioid Response Network	Educational resources and training in the prevention, treatment, and recovery of OUD
Oregon Pain Guidance	Tools and resources for providers and patients to manage pain, OUD, and opioid tapering
Trauma Informed Oregon	Online introductory courses for trauma informed care
Mental Health & Addiction Certification Board of Oregon (MHACBO)	Various live and on-demand addiction education courses
NATIONAL EDUCATION RESOURCES	DESCRIPTION
Providers Clinical Support System (PCSS)	Training courses including SUD 101 and SUD for the Healthcare team
American Society of Addiction Medicine (ASAM)	Live and on demand training resources, including ED specific courses
National Harm Reduction Coalition	Series of self-paced online learning modules for harm reduction strategies and tools

4. COMMUNITY RELATIONSHIPS & REFERRAL NETWORKS

Another commonly cited barrier to OUD treatment initiation in the ED is the lack of outpatient referral partners and the inability of ED staff to ensure patients receive appropriate follow up. Traditionally, a patient may be provided with a name and phone number of a local addiction treatment facility and asked to call when the facility is open to schedule an appointment. Unfortunately with this type of referral system, ED teams are unable to verify that the patient connects with the facility, or that the facility is accessible to the patient and able to meet their individual treatment needs. Given that ED visits often happen outside of outpatient visit hours, **it is especially important that EDs build strong ongoing relationships with their local referral partners, and if possible, utilize an electronic referral system** that will help ensure patients receive the follow-up care they need.

“Connections between acute care services and SUD treatment, including MOUD, are crucial to help mitigate the impact of opioid overdose in Oregon, engage patients in essential services, and save lives”

*- John McIlveen, Ph.D., LMHC
State Opioid Treatment Authority, OHA*

Referral Agreements

It is recommended that EDs establish referral agreements with their local community substance use treatment sites. Referral agreements should include, but are not limited to, the following mutual expectations and responsibilities:

- The best method to send referrals, both during and after business hours
- What information will be included in each referral
- An agreement of how quickly patients will be contacted and seen
- How the outpatient site will report outcomes back to the ED
- Designated point persons at each site
- An ongoing plan and timeline for revisiting the agreement

Some primary care providers (PCPs) also offer ongoing OUD treatment for their patients. EDs may want to check with local PCPs to determine if they are willing to provide medication and support for established patients who have initiated treatment in the ED.

Referral Networks

These electronic referral networks are available in Oregon, and can help connect patients to outpatient treatment services, as well as needed social and community resources.

REFERRAL NETWORKS	DESCRIPTION
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Connect Oregon	Coordinated network of health and social service organizations, capable of automatic referrals, closed-loop communication, network analytics, and EHR integration
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211info Coordination Center	Care Coordinators available in 14 Oregon counties to assist with referrals to health and social services via the Connect Oregon system
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Locating Referral Partners

If you do not have access to an electronic referral network, these resources can help you and your patients find SUD treatment providers and supports in your area:

OREGON PROVIDER & COMMUNITY SUPPORT LOCATOR RESOURCES	DESCRIPTION
<u>OHA SUD Services Directory</u>	Directory of SUD treatment providers by county, including types of services offered
<u>211info</u>	Health and social service directory that is available online, via email, or phone
<u>Oregon Overdose-Related Services & Projects Summary</u>	County specific information on overdose-related services such as opioid treatment programs, pregnancy related supports, and peer support
<u>Coast to Forest</u>	Mental health and substance use resources in rural Oregon, including a provider directory by county

If you need assistance, your local [Coordinated Care Organization \(CCO\)](#) may be able help you develop relationships and referral pathways with community providers in your area.

Telehealth Referrals

Referrals to follow up care delivered via telehealth can improve access and often allows for rapid appointment scheduling. Virtual care can remove transportation barriers, offer expanded options for patients in rural areas, and can be provided through any smartphone. The following telehealth treatment options are available to patients in Oregon:

CLINIC	DESCRIPTION
<u>HRBR Clinic</u>	Low-barrier, on-demand access to people who are interested in cutting back or stopping their alcohol or drug use. Services include MAT, disease screening, peer support, and connections to longer-term care. Accepts any and no insurance. Located at OHSU.
<u>Boulder Care</u>	Comprehensive, long-term virtual telehealth treatment for opioid and alcohol addiction treatment. Services include MAT, peer support, case management, care advocacy, as well as mental and physical health services. Referrals can be made 24/7 via their website.

Note: Some patients may have barriers to care delivered via telehealth, such as those without a smartphone or tablet, without necessary bandwidth or data, without a private place to conduct a visit, or with cultural beliefs that do not align with virtual care.

Establishing Best Practice Workflows

Standardized, clear protocols allow for efficient identification and appropriate care for patients in the ED. **There are three main steps of the OUD management process: (1) patient identification, (2) treatment, and (3) discharge planning.** Each step can be made easier by leveraging an expanded care team and efficient electronic tools that will support both the provider and patient.

Patient identification

There are several different methods for identifying patients who are appropriate for OUD treatment. Options range from OUD screening for all patients to only those experiencing opioid withdrawal.

In 2021, over 90,000 ED visits in Oregon included a diagnosis code related to substance use but did not include an overdose code.²⁶ This indicates that many people that use drugs are visiting the ED for a variety of issues other than overdose and underscores the importance of the ED as a crucial location to recognize patients in need of treatment.

While it is recommended that **ED-initiated buprenorphine is considered for any patient with OUD who is not currently engaged in an MOUD program**, screening all patients may not always be feasible in a busy department. Therefore, each ED should tailor their identification process based on their time and resource capacities. If possible, **screening should be embedded in the electronic health record (EHR) triage workflow**, to ensure it is completed and recorded for all appropriate patients.

Screener Options

The validated questionnaires below may be used to identify patients with OUD, particularly those who do not present with symptoms indicative of drug use. Patients may not answer truthfully if they are concerned about stigma, so it is important let them know they will not be judged on the answers and treatment and support will be offered to them.

SCREENER	DESCRIPTION
SBIRT Brief Screen	For patients 18 and over. Uses a single question that can be asked by any staff member, “How many times in the past year have you used a recreational drug or used a prescription medication for non-medical reasons?”
Drug Abuse Screening Test (DAST)	10 additional questions for patients 18 and over who screen positive on the Brief Screen
CRAFT 2.1+N	For patients 12-17. Answers can indicate a possible substance use disorder. Should be administered when the patient is alone.

Screeners in additional languages are available online at [SBIRT Oregon](#).

Patient Identification Methods

EDs may **use all or some of the following** options to identify patients appropriate for OUD treatment.

- | | |
|-------------------------------------|---|
| ➤ Universal Screening | Patients with OUD may or may not present to the ED with concerns or presentations related to opioid use. ²⁰ A screening questionnaire given at check-in or in the waiting room can identify those who may not otherwise offer the information or who do not realize that help is available in the ED. |
| ➤ Self-Identification | Signs in public spaces or patient rooms can encourage people to self-identify opioid use and convey that treatment and support is available ²⁴ |
| ➤ Drug-related concern | Patients may be asked about opioid use if they present with signs of possible substance use, including the following: ²⁴ <ul style="list-style-type: none">○ Nonfatal overdose○ Abscesses or cellulitis○ Endocarditis○ Hepatitis C or HIV○ Positive urine toxicology testing○ Admitted or signs of alcohol, methamphetamine, or other substance use |
| ➤ Opioid withdrawal symptoms | All patients presenting with symptoms of opioid withdrawal should be asked about opioid use. Buprenorphine is the first line treatment for patients experiencing opioid withdrawal in the ED. Although timely access to continuing treatment is the goal after discharge, the benefit still strongly favors buprenorphine treatment for almost all patients, even if follow-up prescribing cannot be assured. ²⁰ |

OREGON Spotlight: Best Practice Workflows

Oregon Health & Science University's (OHSU's) [**IMPACT Program**](#) is comprised of a team of providers, social workers, nurses, pharmacists, and peers that offer consults for patients with substance use disorder, work collaboratively with clinicians throughout the hospital, and develop resources for the entire community.

Their comprehensive processes include education, written policies and procedures, electronic order sets, use of peer support, specialist consultation, and a low-barrier follow-up clinic.

OHSU Resources: [Includes policies, order sets, a best practice toolkit, and patient education](#)

Education for providers: [Oregon ECHO Network videos](#)

Treatment

Treatment in the ED will vary depending on several factors such as the amount of opioid used, the patient's level of withdrawal, their access to outpatient care, and their readiness to engage in ongoing treatment. In addition to MOUD induction, effective informed consent conversations, the availability of specialty consultations, comprehensive electronic supports, and harm reduction strategies are all key aspects of treatment that can improve the engagement and success of both patients and providers.

Informed Consent²⁷

Prior to initiating buprenorphine, informed consent conversations should be completed with the patient, during which they should be educated on topics such as:

- How buprenorphine works
- Expected results
- Benefits of initiation
- Associated risks
- Safety precautions
- Alternative treatments
- Potential consequences of not receiving any treatment

Informed consent conversations should be patient-centered, meaning the patient is an active participant and their autonomy and choices are respected. Effective informed consent conversations can help to strengthen the patient-provider relationship and help patients feel knowledgeable and empowered to work with the ED team to choose the path that will work best for them.

A written treatment agreement covering the informed consent conversation topics may be provided to the patient. Sample agreements can be found via the resources below:

RESOURCE	DESCRIPTION
Providers Clinical Support System (PCSS)	Buprenorphine information sheets for patients/families and sample treatment agreements
IT MATTTRs™	Sample buprenorphine consent forms and treatment agreements

Specialist Consultation

Consultation with an addiction medicine/psychiatry specialist can be beneficial when treating complex patients. If there is not an available specialist in your system, it is recommended that EDs establish a consultation process with a local treatment organization.

The [OHSU Addiction Consult Line](#) provides free, same day services to clinicians, pharmacists, and other staff in Oregon. It is available Monday-Friday from 8am-5pm (excluding major holidays).

Call **503-494-4567** and ask for the Addiction Consult Line.

Electronic Order Sets

The development of EHR-embedded order sets, clinical decision support tools, and narrative templates can make it easy for providers to adhere to your OUD treatment protocols and ensure patients receive best practice care. Consider adding the following to your order sets: ²⁷

- Buprenorphine dosing
- COWS assessment
- Supportive medications
- Lab tests
- Opioid withdrawal management
- Naloxone at discharge
- Buprenorphine prescription at discharge
- Review of the prescription drug monitoring program (PDMP)

EXAMPLES	DESCRIPTION
OHSU	General buprenorphine induction order set
CA Bridge	Sample ED buprenorphine induction order sets
EMBED	Clinical decision support tool that can be accessed via smartphone, online, or incorporated into the EHR



TIP: X-WAIVERS (i.e., DATA 2000 waiver or MAT waiver)

Prior to 2023, a special DEA certification (X-Waiver) was required for clinicians to prescribe buprenorphine. Depending on the clinician's license type, 8 to 16 hours of additional training was mandatory. In 2021, the training requirements were modified such that prescribers treating 30 or fewer patients at any one time could forego the training, however x-waivers were still needed.

In January 2023, as part of the federal Mainstreaming Addiction Treatment Act, **the X-waiver was eliminated completely**. Effective now, any clinician with a current DEA registration may prescribe buprenorphine for OUD.²⁹ There are no limits on the number of patients a prescriber may treat.

However, the simultaneously passed Medication and Access and Training Expansion Act, requires **all clinicians applying for or renewing their DEA license to complete a one-time, 8-hour training** on treating and managing patients with SUD. Certain providers, such as those board certified in addiction medicine or recent graduates with applicable curriculum, will not have to complete any additional training. This requirement becomes effective in June 2023.³⁰

Harm reduction

Harm reduction strategies aim to reduce the risk of negative consequences of opioid use for patients who may not be ready to quit. Providing knowledge, skills, and resources can help improve quality of life and reduce morbidity.²⁰ Harm reduction techniques include:



Provide compassionate care and an OPEN-DOOR POLICY

All team members in the ED should recognize addiction as a chronic disease and thoughtfully care for patients without bias. Patients who are not ready to initiate medication should be advised that the ED providers are there to help them and will be available if and when the patient becomes ready to engage in treatment.



Distribute NALOXONE

Naloxone kits or prescriptions, as well as accompanying education should be given to all patients at risk for an opioid overdose. See Tip Sheet on page 22 for more information.



Offer LAB SCREENINGS²⁰

Encourage patients with OUD to undergo testing for Hepatitis C, HIV, and pregnancy



Teach SAFE INJECTION PRACTICES²⁰

- **Avoid using alone.** If you overdose, you want someone to help.
- **Be cautious if you haven't used in a while.** You're more likely to overdose.
- **Avoid mixing.** Many overdoses happen when opioids are mixed with other drugs like benzos, methadone, antidepressants, and/or alcohol.
- **Always do a test shot** to make sure a new batch isn't too strong.
- **Make an overdose plan.** Be prepared with naloxone and have a phone on hand in case you need to call 911.
- **Don't be afraid to call 911.** If you're with someone who you think is overdosing, call 911. The law provides substantial protection from prosecution.
- **Always use new equipment, and never share equipment.** Many communities anonymously provide free syringes and drug use equipment.
- **Never lick needles**, use **sterile water**, and **discard cotton** after every use.
- **Refer to a needle exchange program** if appropriate

Discharge planning

Patients discharged after buprenorphine induction should have a clear plan for accessing follow-up care. Upon discharge, it is recommended that patients are provided with the following:

- 1. Referral:** An efficient, robust link to ongoing outpatient treatment will significantly increase a patient's likelihood of success. See "Community Referral Networks and Relationships" on pages 8-9 for more information on how to identify and establish referral arrangements. Ideally, a patient will have an appointment scheduled prior to discharge. As this is not always feasible, an electronic referral or direct communication with the outpatient facility from the ED can help ensure an appointment is made.
- 2. Buprenorphine prescription:** Any clinician with a current DEA registration that includes Schedule III authority may provide a prescription for buprenorphine to bridge the gap until outpatient care is established. The ability to access the pharmacy and pay for the prescription should be discussed with the patient.
- 3. Naloxone:** All patients at risk for an opioid overdose should be discharged with a take-home naloxone rescue kit or a naloxone prescription. See Tip Sheet on page 22 for more information.
- 4. Discharge Instructions:** Should be written at a 5th grade reading level in the patient's primary language if possible. Visual guidance (e.g., pictures or diagrams) may also be helpful. Instructions may include:²⁰
 - Education about opioid use disorder
 - How buprenorphine works and how to administer it
 - Signs of opioid overdose and withdrawal
 - Warnings of concurrent use with sedatives
 - Guidance on safe medication storage
 - Harm reduction strategies
 - Use of naloxone
 - When to return to the ED
 - Details of the follow-up plan, including name, location, and contact information for the outpatient treatment facility

RESOURCE	DESCRIPTION
ColoradoMAT	Sample discharge instructions
CA Bridge	Sample discharge instructions, available in multiple languages and includes a customizable template

Enhanced care teams

Trained support staff available to help patients understand and navigate their care are a crucial element of any OUD treatment program. These individuals can provide **essential support for the entire OUD treatment process**, from triage through (and even after) discharge.

Support staff may have several different titles. The most common are **care navigators, care managers, care coordinators, and substance use navigators**. Varied licenses and certifications are appropriate for this work including social workers, nurses, and traditional health care workers (THW).

Peer support specialists can also be extremely helpful in this role. Peer support specialists are trained traditional health workers that are in long-term recovery from a substance use disorder. This shared experience can help promote engagement and trust with patients.

While their titles, backgrounds, and job functions may look slightly different in each hospital, all of these care team members can provide logistical support, emotional support, and advocacy for patients, while minimizing the workload and time commitment for ED providers.³¹ It is recommended that they are available 24/7 to always ensure high quality care for patients and relief for other ED staff.

Support staff may provide the following types of services for patients with OUD in the ED:^{32,33}

- Conduct OUD screening
- Identify and engage patients with OUD and explain treatment options
- Assist with informed consent conversations
- Discuss harm reduction
- Address any barriers to treatment
- Identify and coordinate referrals to outpatient OUD treatment
- Screen for and connect to additional needed services such as physical care, mental health care, and community resources for social needs (such as housing, food, or transportation)
- Assist patients with signing up for health insurance if needed
- Arrange for infectious disease testing or treatment
- Provide or arrange for harm reduction supplies
- Deliver discharge instructions and education
- Remain available for the patient to contact after discharge if needed

OREGON RESOURCES	DESCRIPTION
Prime+ Peer Services	Peer support services available to EDs in 24 counties in Oregon. Available at no cost to the hospital. Peers have oversight and ongoing training provided by OHA. Peers may be embedded in the ED or contacted to arrive as needed.
Oregon Health Authority (OHA) THW Training Programs	Training programs available for Community Health Workers, Peer Wellness Specialists, Personal Health Navigators, and Peer Support Specialists
NATIONAL RESOURCES	DESCRIPTION
CA Bridge Substance Use Navigation Toolkit	Comprehensive guide for implementing, performing, and sustaining addiction support staff in the ED. Includes resources and patient materials.



OREGON Spotlight: Enhanced Care Teams

“In **St. Charles Bend/Redmond** emergency department, **BestCare’s peer team** often works with patients with opioid use disorder, having just experienced an overdose, or other medical issues. Our peers can help the patient with immediate access to resources such as rapid access intakes into detox, medication supported recovery (MSR) or medications for addiction treatment (MAT) clinics, Narcan, as well as basic needs. These basic needs include transportation, food, tents, sleeping bags, and referrals to PCP’s and mental health services.

Our lived experience as peers helps us to build a unique connection with the patients we serve that leads to long lasting trust and rapport. The patients we see in the ED are often not ready to quit but continue to communicate with our peers for harm reduction support, and basic needs. For those patients that are ready to stop using opioids, we are able to complete the intake and assessment process to get them into SUD treatment almost immediately.”

- *Lynette Espinoza*
BestCare Peer Supervisor

“Since the passing of HB4143, **Salem Hospital** Emergency Department has been utilizing PRIME+ team for patients who arrive after an overdose, in withdrawal of opiates, and/or seeking treatment. In collaboration with the ED providers, Salem Hospital triggers the PRIME+ team and collects baseline labs to set the patient on the path of recovery. For those patients in active withdrawal, the ED providers can also prescribe the first dose of buprenorphine to decrease symptoms. **The PRIME+ team responds 24/7 to the emergency department and meets the patient at whatever stage of recovery they are currently in** supporting them through next steps toward health. “

- *Nancy Bee, RN, MBA*
ED Director, Salem Hospital

Treatment Recommendations

The treatment recommendations found in this section are also intended to be able to serve as short, stand-alone documents that can be used by ED personnel as quick-guides to manage patients that need buprenorphine or naloxone.

We encourage hospitals to share these not only with those contributing to the development of an OUD treatment program, but with all members of the ED team that participate in the care of patients with substance use disorders.

For ease of sharing, you can also find these documents available separately on the OHLC website:

- [Tips for Treating Opioid Use Disorder with Buprenorphine](#)
- [Tips for Dispensing Naloxone](#)



TIP: Polysubstance Use

People with OUD often take other non-prescribed substances, leading to increased patient complexity and severity.^{34,35,36} Unfortunately, recent evidence suggests that people with polysubstance use are significantly less likely to receive MOUD initiation than those without co-occurring SUD.^{35,36,37}

The American Society of Addiction Medicine cautions that **concurrent use of other drugs should not exclude or delay a patient from receiving treatment for OUD, including medications**. Patients should be evaluated for the need for a higher level of care and co-occurring substance use should be addressed. Even if a patient is unable or unwilling to obtain care or treatment for other substance use, OUD treatment should proceed. Failure to initiate or continue OUD treatment due to co-occurring substance use may increase the risk of overdose, accidents, or other poor outcomes.³⁸

Patients can be successfully treated for OUD, even in the presence of other substance use. Although the patient may be more complex, OUD treatment can still significantly reduce their risk for harm and should not be withheld,³⁹ including from patients taking benzodiazepines or other central nervous system depressants.⁴⁰

Best Practices in Emergency Medicine – Tips For Treating Opioid Use Disorder with Buprenorphine

THE SITUATION

Opioid use disorder (OUD) is treatable with medication and support, but rates in the US have reached epidemic proportions. Emergency departments (EDs) are uniquely positioned to offer buprenorphine, an evidence-based life-saving treatment, when patients are most open to receiving it, but few currently offer it.

SAMPLE BUPRENORPHINE INITIATION WORKFLOW*

STEP 1 Identify patients	- Those with acute opioid withdrawal, <i>or</i>; - With OUD based on DSM-5 criteria (see page 2), <i>and</i>; - Are ready to begin treatment
STEP 2 Determine time since last opioid	Patients should meet minimum time requirements prior to induction: - Short-acting (e.g., oxycodone, heroin): at least 8-12 hours - Extended release (e.g., oxycontin): at least 24 hours - Long-acting (e.g., methadone): at least 72 hours - Fentanyl: precipitated withdrawals can occur after 72 hours. If use is suspected, base induction start time of level of withdrawal (step 3)
STEP 3 Identify level of withdrawal	Use the Clinical Opiate Withdrawal Scale (COWS) to measure withdrawal severity https://www.mdcalc.com/calc/1985/cows-score-opiate-withdrawal
STEP 4 Initiate treatment	If fentanyl is suspected and COWS ≥ 16 with 2 or more objective signs of withdrawal (e.g., pupils $> 3\text{mm}$, heart rate $> 100\text{ bpm}$), give 8-16mg SL. Observe for 45-60 minutes. If fentanyl is not suspected and COWS ≥ 10 with at least one objective sign of withdrawal give 2-8 mg SL based on severity of symptoms. Observe for 45-60 minutes. - Use higher dose if patient is a heavy opioid user or COWS ≥ 13 - If patient is on long-acting opioids, wait until COWS ≥ 13 If COWS threshold is not met, consider home induction, observe until COWS is met, or refer to outpatient treatment If the patient has completed withdrawal (several days to weeks since last opioid), but cravings persist, give 2-8mg SL. Observe for 30-60 minutes.
STEP 5 Subsequent dosing	- If withdrawal symptoms are still present after observation, give second dose and observe - Give supportive medications as needed (see page 2) - Titrate until symptoms resolve, up to 32mg maximum. High doses are only recommended for those not at risk for respiratory depression
STEP 6 Discharge Planning	- With care navigator, refer to outpatient treatment as soon as possible - With care navigator, provide harm reduction strategies such as take-home naloxone kit, overdose education, reproductive health counseling, safe injection practices - Provide prescription for buprenorphine/naloxone 16mg-24mg (higher doses for those using fentanyl) for 1-2 weeks, or until date of scheduled follow up appointment

*Significant variations exist within published clinical guidance. Patient presentation, the substance used, how much, and for how long will all affect the level of withdrawal needed to be reached, as well as the dose amounts given. These recommendations are not meant to be prescriptive in any manner, but to serve as a foundation to build your own protocols. Providers should continue to use clinical judgement when initiating buprenorphine.

OUD DIAGNOSIS CRITERIA¹

- | | | |
|--|--|---|
| <ol style="list-style-type: none">1. Often using in larger amounts or for longer than intended2. Persistent desire or unsuccessful efforts to reduce or control use3. Great deal of time spent obtaining, using, or recovering4. Craving or strong desire to use5. Failure to fulfil major work/school/home obligations due to use6. Social/interpersonal problems caused or exacerbated by use | <ol style="list-style-type: none">7. Important activities stopped or reduced because of use8. Recurrent use in physically hazardous situations9. Physical or psychological problems caused/exacerbated by use10. Tolerance (need for increased amounts of opioids to achieve desired effect or diminished effect with continued use of the same amount)11. Withdrawal (opioid withdrawal syndrome or taking the substance to relieve or avoid withdrawal symptoms) | MILD: 2-3 criteria
MODERATE: 4-5
SEVERE: 6 or more |
|--|--|---|

SUPPORTIVE MEDICATIONS

Supportive medications, such as the following, may be used to manage withdrawal symptoms:

Nausea/vomiting	ondansetron, promethazine	Diarrhea	loperamide
Myalgias	acetaminophen, ibuprofen	Muscle spasms	tizanidine, methocarbamol
Insomnia	trazodone	Abdominal cramping	hyoscyamine
Restlessness/agitation*	gabapentin, lorazepam, diazepam	Anxiety/dysphoria	hydroxyzine, diphenhydramine, clonidine

*If using a benzodiazepine, consider observing for 1-2 hours prior to discharge

ADDITIONAL CONSIDERATIONS

- **Concurrent sedative use:** Use caution if the patient has alcohol intoxication, uses benzodiazepines or other sedatives. Consider observation until sedative effects have subsided.
- **Precipitated withdrawal:** If withdrawal symptoms suddenly and significantly worsen after the first dose, suspect buprenorphine precipitated withdrawal (BPW). There are two different pathways to treat BPW, one is to treat the withdrawal symptoms and abort the induction, the second is to aggressively continue with higher doses of buprenorphine. There is a lack of evidence to recommend one over the other. Consult an addiction medicine specialist if possible and consider admitting the patient for observation.
- **Complex patients:** Consider a specialist consult for complex inductions such as methadone use, serious acute medical illness, altered mental status, planned surgeries, chronic opioid therapy for pain, moderate to severe liver disease, or respiratory failure.
- **Pregnancy:** While buprenorphine is safe to use in pregnancy, withdrawal can increase the risk of miscarriage. Use buprenorphine mono-product and consider admission to L&D with fetal monitoring.
- **Formulations:** Sublingual (SL) films and tablets are recommended for use in the ED. IV buprenorphine may be used if vomiting prevents SL administration. Buprenorphine with or without naloxone may be administered in the ED. Naloxone is traditionally added as an abuse deterrent as it has low bioavailability through SL absorption, but is expected to cause withdrawal if crushed and injected. However, recent evidence suggests naloxone may not be an effective deterrent.²
- **Micro-induction:** There is emerging acceptance of rapid micro-induction of buprenorphine to minimize withdrawal symptoms and risk of precipitated withdrawal. Providers are encouraged to learn about this technique. However, it is not currently recommended in the ED.³
- **Education:** Advise patients to keep buprenorphine in safe storage and away from children.

ABOUT FENTANYL

Due to the high prevalence of Fentanyl in the current illicit drug supply, **use should be suspected unless it can be confirmed otherwise**. If available, rapid urine screens can be used to assess for use.

Recent evidence suggests that in patients with long-term or heavy fentanyl use, **precipitated opioid withdrawal can occur more often with buprenorphine induction**⁴, even after waiting several days after use.⁵ This is thought to be caused by fentanyl's unique pharmacological properties:⁶

- Fentanyl has **high lipophilicity**, so is stored and gradually released from fat tissue
- Fentanyl has a considerably **extended renal clearance** time when compared to other short-acting opioids (up to several weeks)

In addition to the amount and length of time that fentanyl has been taken, physical dependence, genetics, comorbidities, polysubstance use, and more may affect the buprenorphine threshold dose that will precipitate withdrawals. Therefore, although protocols may serve as a helpful guide, providers are encouraged to **adapt their approach to each individual patient**.⁵ Expert consultation should be sought if needed.

RESOURCES

- Management of Opioid Use in the Emergency Department: A White Paper Prepared for the American Academy of Emergency Medicine. <https://www.aaem.org/UserFiles/file/AAEMOUDWhitePaperManuscript.pdf>
- CA Bridge: Buprenorphine (Bup) Emergency Department Quick Start. <https://cabridge.org/resource/buprenorphine-bup-hospital-quick-start/>
- ACEP Buprenorphine Use in the Emergency Department (BUPE) Tool. <https://www.acep.org/patient-care/bupe/>
- ACEP E-Qual Opioids Toolkit. <https://www.acep.org/administration/quality/equal/emergency-quality-network-e-equal-e-equal-opioid-initiative/e-equal-opioid-toolkit/>
- SAMHSA Use of Medication-Assisted Treatment in Emergency Departments. https://store.samhsa.gov/sites/default/files/SAMHSA_Digital_Download/pep21-pl-guide-5.pdf

REFERENCES

1. American Psychiatric Association (2013). Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition. Washington, DC, American Psychiatric Association page 541
2. Blazes, C. K., & Morrow, J. D. (2020). Reconsidering the Usefulness of Adding Naloxone to Buprenorphine. *Frontiers in psychiatry*, 11, 549272. <https://doi.org/10.3389/fpsyt.2020.549272>
3. Greenwald, M. K., Herring, A. A., Perrone, J., Nelson, L. S., & Azar, P. (2022). A Neuropharmacological Model to Explain Buprenorphine Induction Challenges. *Annals of emergency medicine*, 80(6), 509–524. <https://doi.org/10.1016/j.annemergmed.2022.05.032>
4. Varshneya, N. B., Thakrar, A. P., Hobelmann, J. G., Dunn, K. E., & Huhn, A. S. (2022). Evidence of Buprenorphine-precipitated Withdrawal in Persons Who Use Fentanyl. *Journal of addiction medicine*, 16(4), e265–e268. <https://doi.org/10.1097/ADM.0000000000000922>
5. Silverstein, S. M., Daniulaityte, R., Martins, S. S., Miller, S. C., & Carlson, R. G. (2019). "Everything is not right anymore": Buprenorphine experiences in an era of illicit fentanyl. *The International journal on drug policy*, 74, 76–83. <https://doi.org/10.1016/j.drugpo.2019.09.0>
6. Huhn, A. S., Hobelmann, J. G., Oyler, G. A., & Strain, E. C. (2020). Protracted renal clearance of fentanyl in persons with opioid use disorder. *Drug and alcohol dependence*, 214, 108147. <https://doi.org/10.1016/j.drugalcdep.2020.108147>

Best Practices in Emergency Medicine – Tips For Dispensing Naloxone

THE SITUATION

Community overdose education and naloxone distribution programs have been shown to decrease overdose deaths, and not increase opioid use. **The emergency department (ED) is an ideal setting for opioid overdose death prevention through the distribution or prescribing of naloxone rescue kits**, overdose prevention and response education.¹

AT RISK PATIENTS

Any patient at risk for an opioid overdose should be dispensed or prescribed a naloxone rescue kit

Patients at risk can include, but are not limited to, the following:^{2,3,4,5}

- **Any patient with substance use disorder (SUD)**, because all illicit substances should be assumed to contain fentanyl, including non-opioids
- Received emergency care involving **opioid intoxication, overdose, or withdrawal**
- History of, suspected, or current **opioid abuse** or nonmedical opioid use
- Prescribed or initiated treatment of **buprenorphine or methadone**
- Receiving an **opioid prescription** of > 50 mg morphine equivalents per day
- Receiving an opioid prescription for pain and:
 - Has respiratory, renal, hepatic, or cardiac disease
 - Uses alcohol, benzodiazepines, or other sedatives
 - Has poorly controlled depression
- **Resuming opioid use** after a period of abstinence (including incarceration)
- Has difficulty accessing emergency services (e.g., rural residence)
- Requested by patient or concerned friend/family member

NALOXONE RESCUE KITS

If possible, distribution of a **take-home kit is preferred** over a prescription, to ensure access and remove possible pharmacy cost or availability barriers.³ Naloxone kits should include:⁵

- 2 doses of naloxone, preferably in intranasal (IN) formulation, which is easy to use and often covered by insurance
 - Note: Generic intramuscular (IM) formulations are low cost and may be appropriate for those with experience drawing and injecting. Auto-injector formulations are costly and often not covered by health insurers.
- Instructions on use and other educational handouts (see p.2)
- May include alcohol swabs, gloves, and/or face shields

WORKFLOW RECOMMENDATIONS

1. Incorporate a **naloxone order set** into your electronic health record (EHR), which will be suggested for any patient in the ED with an SUD-related diagnosis
2. Authorize **standing orders for naloxone** distribution, so kits can be dispensed by care team members such as nurses or care navigators

PATIENT EDUCATION

Naloxone training should be provided in person to the patient and their friends/family, and should include discussion of the following:^{2,3,6}

- ✓ Overdose risk factors
- ✓ Signs and symptoms of opioid overdose
- ✓ Importance of immediate activation of 911
- ✓ Instructions on how to administer naloxone
- ✓ Naloxone effects and side effects
- ✓ Instruction on compression only CPR or information on local lay-person CPR training

Patient Education Handouts and Resources

- Naloxonesaves.org - [How to use a naloxone kit](#)
- Prescribetoprevent.org - [Opioid safety and how to use naloxone](#), [How to prevent an overdose](#)
- Veteran's Health Administration - [Overdose Prevention and Reversing an Overdose](#)
- American Heart Association - [CPR course locator](#)

ACCESSING NALOXONE

The cost of dispensing take-home naloxone rescue kits can be a barrier. The following resources may be able to help provide assistance or guidance:

Coordinated Care Organizations	Contact your local CCO , they may have funding available for overdose prevention programs
Oregon Community Hospital Benefit	Talk to hospital administration about using Community Benefit funding to support naloxone distribution
Grants	Investigate grants from organizations such as SAMHSA , Central Oregon Pain Guide , and Rural Communities Opioid Response Program
Community partners	Connect with local law enforcement, syringe exchange programs, criminal justice department, county health department, etc. to discuss opportunities for partnership or funding
Pharmacy partners	Collaborate with your hospital pharmacy to discuss opportunities to supply the ED with naloxone kits
Save Lives Oregon	Save Lives Oregon provides harm reduction supplies, including naloxone, to community based organizations, health clinics, tribes, first responders and jails
Max's Mission	Max's Mission provides free naloxone and harm reduction supplies in Josephine, Jackson, and Klamath counties

REFERENCES/RESOURCES

1. ACEP Buprenorphine Use in the Emergency Department (BUPE) Tool. <https://www.acep.org/patient-care/bupe/>
2. SAMHSA Opioid Overdose Prevention TOOLKIT <https://store.samhsa.gov/sites/default/files/d7/priv/sma18-4742.pdf>
3. ACEP & TIPS: Emergency Department Naloxone Distribution: Key Considerations and Implementation Strategies <https://prescribetoprevent.org/wp2015/wp-content/uploads/TIPSWWhitePaper.pdf>
4. ACEP Policy Statement: Naloxone Prescriptions by Emergency Physicians <https://www.acep.org/globalassets/new-pdfs/policy-statements/naloxone-prescriptions-by-emergency-physicians.pdf>
5. PCSS MAT Training: Prescribing Naloxone to Patients for overdose Reversal <http://pcssnow.org/wp-content/uploads/2016/08/Prescribing-Naloxone-to-Patients-for-Overdose-Reversal.pdf>
6. ACEP Policy Statement: Naloxone Access and Utilization for Suspected Opioid Overdoses <https://www.acep.org/globalassets/new-pdfs/policy-statements/naloxone-access-and-utilization-for-suspected-opioid-overdoses.pdf>

Maintaining Endurance

Sustaining a successful OUD treatment program in the ED relies upon a systematized process of assessment, feedback, and adaptation. With adequate support and resources, OUD treatment can become the standard of care and integrated into the culture of the department.

Continual quality improvement

OUD treatment programs should continue to be evaluated and refined over time. Collecting, analyzing, and sharing data is an essential part of understanding the impact and identifying areas in need of improvement.²⁴ As much as possible, these activities should be incorporated into already established quality improvement processes, to minimize administrative burden.

1. COLLECT DATA

To understand the impact of your work most thoroughly, gather data from several sources:

- **Quantitative data** is ideally obtained from the EHR or other electronic system(s). Diagnosis codes, pharmacy records, standard template fields, order sets, etc. can all be data sources.
- **Qualitative data** is also important. Gather information about satisfaction, efficiencies, and barriers from providers, team members, and patients.
- **Follow-up data** should be collected whenever possible. Peer navigators or care coordinators can track referral outcomes by outreaching to outpatient sites or patients, or directly from outpatient sites in accordance with a referral agreement (see page 8).

2. TRACK MEASURES

Using the data collected, track a set of measures over time over time. For example:⁴¹

- Number of patients seen with an OUD-related diagnosis
- Number of patients administered buprenorphine
- Number of patients prescribed buprenorphine
- Number of patients given take-home naloxone
- Number of patients prescribed naloxone
- ED visit frequency of patients before and after treatment initiation
- Percent of patients with OUD referred to outpatient treatment
- Number of patients engaged in treatment 30 days after ED initiation

3. SHARE OUTCOMES

Sharing data summaries with the ED team and hospital leadership can help identify areas for improvement and help to build support for the program. In addition to measure results, follow-up information, including **patient success stories** should be shared.^{15,17} These personal accounts from patients, family, or staff can serve to boost morale and reduce bias towards patients with substance use disorders. ED staff often never get to hear about the longer-term differences they are making in individual lives, providing them with this information may help improve motivation and reduce care team burn-out.

Future needs in Oregon

Managing OUD in the ED successfully takes a team of trained, engaged, and persistent leaders and front-line workers. We hope the information and resources provided in this Toolkit assists your organization in effectively building or maintaining your OUD treatment program. We believe that every ED in Oregon can play an integral part in helping patients with substance use disorder get the care they need. We recognize, however, that barriers remain that can make OUD treatment programs difficult, costly, or less efficient than they should be.

Below are our primary recommendations for future focus areas for Oregon, based on findings from [our survey](#) and discussions with providers:



Increase post-discharge treatment options – In our survey of Oregon EDs, the lack of referral options for continued treatment is the most commonly cited barrier to initiating treatment in the ED, particularly for rural hospitals.

We recommend continued focus on the need to fund and develop additional outpatient treatment options, including expansion of telehealth-based treatment, as well as inpatient facilities that can medically manage complex SUD patients.



Develop access to expert consultation – Rising rates of mental health disorders, polysubstance use, and the emergence of potent new illicit drugs have significantly increased the complexity of many patients seeking help from our EDs. Providers across the state need reliable access to addiction medicine experts who can help guide treatment for these patients.

We recommend the development of regional addiction medicine consult lines, available 24/7 for all providers treating patients with substance use.



Address cost of naloxone take-home kits – While some EDs are providing prescriptions or resources for obtaining free naloxone upon discharge, taking home naloxone is the only way to ensure patients actually receive this life-saving medication. However, the cost for hospitals to dispense naloxone directly to patients often prevents them from doing so.

We recommend statewide collaborative action to secure sustainable funding and reduce the cost of naloxone distribution through the ED, as well as educate all parties on the benefit of continued investment in take-home naloxone.



Expand access to peer support – Despite data showing that the use of peer support leads to decreased illicit drug use, lower ED use, and increased rates of employment, services are currently underutilized and many patients, as well as ED staff, are unaware of their benefits.

We recommend that all Oregon hospitals embed peer support personnel into the ED, allowing low-barrier access to services for patients with substance use disorders.

“We don’t currently have a system that can produce reliable results. The work before us is to make sure that any patient walking into any ED in Oregon has an equal opportunity for success in managing their OUD.”

*- Nick Kashey, MD, MPH
Clinical VP, Population Health, Legacy Health*

References

1. Mental Health & Addiction Certification Board of Oregon. (2021, December). *Oregon Data extracted from the national Survey on Drug Use and Health, released December 2021*. <https://drive.google.com/file/d/1WYJOaDSrvHeKan2A0rOHqW9dsacrDvPP/view>
2. Oregon Health Authority. (n.d.). *Prescribing and Drug Overdose Data Dashboard*. Opioid Overdose and Misuse: Public Health Division. <https://www.oregon.gov/oha/ph/preventionwellness/substanceuse/opioids/pages/data.aspx>
3. Morgan, J. R., Schackman, B. R., Weinstein, Z. M., Walley, A. Y., & Linas, B. P. (2019). Overdose following initiation of naltrexone and buprenorphine medication treatment for opioid use disorder in a United States commercially insured cohort. *Drug and alcohol dependence*, 200, 34–39. <https://doi.org/10.1016/j.drugalcdep.2019.02.031>
4. D'Onofrio, G., O'Connor, P. G., Pantalon, M. V., Chawarski, M. C., Busch, S. H., Owens, P. H., Bernstein, S. L., & Fiellin, D. A. (2015). Emergency department-initiated buprenorphine/naloxone treatment for opioid dependence: a randomized clinical trial. *JAMA*, 313(16), 1636–1644. <https://doi.org/10.1001/jama.2015.3474>
5. Bogan, C., Jennings, L., Haynes, L., Barth, K., Moreland, A., Oros, M., Goldsby, S., Lane, S., Funcell, C., & Brady, K. (2020). Implementation of emergency department-initiated buprenorphine for opioid use disorder in a rural southern state. *Journal of substance abuse treatment*, 112S, 73–78. <https://doi.org/10.1016/j.jsat.2020.02.007>
6. Reuter, Q., Smith, G., McKinnon, J., Varley, J., Jouriles, N., & Seaberg, D. (2020). Successful Medication for Opioid Use Disorder (MOUD) Program at a Community Hospital Emergency Department. *Academic emergency medicine : official journal of the Society for Academic Emergency Medicine*, 27(11), 1187–1190. <https://doi.org/10.1111/acem.13921>
7. D'Onofrio, G., Chawarski, M. C., O'Connor, P. G., Pantalon, M. V., Busch, S. H., Owens, P. H., Hawk, K., Bernstein, S. L., & Fiellin, D. A. (2017). Emergency Department-Initiated Buprenorphine for Opioid Dependence with Continuation in Primary Care: Outcomes During and After Intervention. *Journal of general internal medicine*, 32(6), 660–666. <https://doi.org/10.1007/s11606-017-3993-2>
8. Busch, S. H., Fiellin, D. A., Chawarski, M. C., Owens, P. H., Pantalon, M. V., Hawk, K., Bernstein, S. L., O'Connor, P. G., & D'Onofrio, G. (2017). Cost-effectiveness of emergency department-initiated treatment for opioid dependence. *Addiction (Abingdon, England)*, 112(11), 2002–2010. <https://doi.org/10.1111/add.13900>
9. Olfson, M., Schoenbaum, M., & Goldman-Mellor, S. (2020). Risks of Mortality Following Nonfatal Intentional and Unintentional Opioid Overdoses. *JAMA psychiatry*, 77(11), 1191–1193. <https://doi.org/10.1001/jamapsychiatry.2020.1045>
10. Weiner, S. G., Baker, O., Bernson, D., & Schuur, J. D. (2020). One-Year Mortality of Patients After Emergency Department Treatment for Nonfatal Opioid Overdose. *Annals of emergency medicine*, 75(1), 13–17. <https://doi.org/10.1016/j.annemergmed.2019.04.020>
11. Goldman-Mellor, S., Olfson, M., Lidon-Moyano, C., & Schoenbaum, M. (2020). Mortality Following Nonfatal Opioid and Sedative/Hypnotic Drug Overdose. *American journal of preventive medicine*, 59(1), 59–67. <https://doi.org/10.1016/j.amepre.2020.02.012>
12. Larochelle, M. R., Bernson, D., Land, T., Stopka, T. J., Wang, N., Xuan, Z., Bagley, S. M., Liebschutz, J. M., & Walley, A. Y. (2018). Medication for Opioid Use Disorder After Nonfatal Opioid Overdose and Association With Mortality: A Cohort Study. *Annals of internal medicine*, 169(3), 137–145. <https://doi.org/10.7326/M17-3107>
13. Kilaru, A. S., Xiong, A., Lowenstein, M., Meisel, Z. F., Perrone, J., Khatri, U., Mitra, N., & Delgado, M. K. (2020). Incidence of Treatment for Opioid Use Disorder Following Nonfatal Overdose in Commercially Insured Patients. *JAMA network open*, 3(5), e205852. <https://doi.org/10.1001/jamanetworkopen.2020.5852>
14. Health Share. (2020, June). *Health Share OUD Diagnosis Settings and Treatment, June 2020*.
15. Im, D. D., Chary, A., Condella, A. L., Vongsachang, H., Carlson, L. C., Vogel, L., Martin, A., Kunzler, N., Weiner, S. G., & Samuels-Kalow, M. (2020). Emergency Department Clinicians' Attitudes Toward Opioid Use Disorder and Emergency Department-initiated Buprenorphine Treatment: A Mixed-Methods Study. *The western journal of emergency medicine*, 21(2), 261–271. <https://doi.org/10.5811/westjem.2019.11.44382>
16. Zuckerman, M., Kelly, T., Heard, K., Zosel, A., Marlin, M., & Hoppe, J. (2021). Physician attitudes on buprenorphine induction in the emergency department: results from a multistate survey. *Clinical toxicology (Philadelphia, Pa.)*, 59(4), 279–285. <https://doi.org/10.1080/15563650.2020.1805461>
17. Hawk, K. F., D'Onofrio, G., Chawarski, M. C., O'Connor, P. G., Cowan, E., Lyons, M. S., Richardson, L., Rothman, R. E., Whiteside, L. K., Owens, P. H., Martel, S. H., Coupet, E., Jr, Pantalon, M., Curry, L., Fiellin, D. A., & Edelman, E. J. (2020). Barriers and Facilitators to Clinician Readiness to Provide Emergency Department-Initiated Buprenorphine. *JAMA network open*, 3(5), e204561. <https://doi.org/10.1001/jamanetworkopen.2020.4561>
18. Substance Abuse and Mental Health Services Administration. (2021). *Use of Medication-Assisted Treatment in Emergency Departments*. <https://store.samhsa.gov/sites/default/files/pep21-pl-guide-5.pdf>
19. Hawk, K., Hoppe, J., Ketcham, E., LaPietra, A., Moulin, A., Nelson, L., Schwarz, E., Shahid, S., Stader, D., Wilson, M. P., & D'Onofrio, G. (2021). Consensus Recommendations on the Treatment of Opioid Use Disorder in the Emergency Department. *Annals of emergency medicine*, 78(3), 434–442. <https://doi.org/10.1016/j.annemergmed.2021.04.023>
20. Strayer, R. J., Hawk, K., Hayes, B. D., Herring, A. A., Ketcham, E., LaPietra, A. M., Lynch, J. J., Motov, S., Repanshek, Z., Weiner, S. G., & Nelson, L. S. (2020). Management of Opioid Use Disorder in the Emergency Department: A White Paper Prepared for the American Academy of Emergency Medicine. *The Journal of emergency medicine*, 58(3), 522–546. <https://doi.org/10.1016/j.jemermed.2019.12.034>
21. Fockele, C. E., Duber, H. C., Finegood, B., Morse, S. C., & Whiteside, L. K. (2021). Improving transitions of care for patients initiated on buprenorphine for opioid use disorder from the emergency departments in King County, Washington. *Journal of the American College of Emergency Physicians open*, 2(2), e12408. <https://doi.org/10.1002/emp2.12408>
22. Schoenfeld, E. M., Soares, W. E., Schaeffer, E. M., Gitlin, J., Burke, K., & Westafer, L. M. (2022). "This is part of emergency medicine now": A qualitative assessment of emergency clinicians' facilitators of and barriers to initiating buprenorphine. *Academic emergency medicine : official journal of the Society for Academic Emergency Medicine*, 29(1), 28–40. <https://doi.org/10.1111/acem.14369>

23. Lowenstein, M., Kilaru, A., Perrone, J., Hemmons, J., Abdel-Rahman, D., Meisel, Z. F., & Delgado, M. K. (2019). Barriers and facilitators for emergency department initiation of buprenorphine: A physician survey. *The American journal of emergency medicine*, 37(9), 1787–1790. <https://doi.org/10.1016/j.ajem.2019.02.025>
24. CA Bridge. (2020, September). *Blueprint for Hospital Opioid Use Disorder Treatment*. <https://cabridge.org/resource/blueprint-for-hospital-opioid-use-disorder-treatment/>
25. CA Bridge. (2022, September). *Guide: SUN & Clinic Champion Collaboration*. <https://cabridge.org/resource/substance-use-navigator-clinician-champion-collaboration/>
26. Oregon Health Authority, Public Health Division. (2022, September). *Opioids and the Ongoing Drug Overdose Crisis in Oregon*. https://sharedsystems.dhsosha.state.or.us/DHSForms/Served/1e2479_22.pdf?utm_medium=email&utm_source=govdelivery
27. Stocking, J. T. (2020). Buprenorphine: Medication -Assisted Treatment: The Role of Informed Consent. *Health Law and Policy Brief*, 14(1), <https://digitalcommons.wcl.american.edu/hlp/vol14/iss1/3>
28. CA Bridge. (2020, September). *Guide: Clinical Considerations for Order Sets*. <https://cabridge.org/resource/clinical-considerations-for-order-sets/>
29. Substance Abuse and Mental Health Services Administration. (2023, January 12). *Removal of DATA Waiver (X-Waiver) Requirement Medication Assisted Treatment*. <https://www.samhsa.gov/medication-assisted-treatment/removal-data-waiver-requirement>
30. United States Congress. (2021, March 18). *H.R.2067 MATE Act of 2021*. 117th Congress. <https://www.congress.gov/bill/117th-congress/house-bill/2067>
31. Kelly, T., Hoppe, J. A., Zuckerman, M., Khoshnoud, A., Sholl, B., & Heard, K. (2020). A novel social work approach to emergency department buprenorphine induction and warm hand-off to community providers. *The American journal of emergency medicine*, 38(6), 1286–1290. <https://doi.org/10.1016/j.ajem.2019.12.038>
32. CA Bridge. (2022, September). *Substance Use Navigation Toolkit*. <https://cabridge.org/resource/substance-use-navigation-toolkit/>
33. Prime+. (n.d.). *Prime+: Peer Recovery Initiated in Medical Establishments + HCV/HIV Testing and Linkage to Treatment*. <https://www.oregon.gov/ohcs/get-involved/Documents/committees/HTF/11-12-2021-Oregon%20PRIME%20Plus%20-%20Overview-1%20page.pdf>
34. Saloner, B., Whitley, P., LaRue, L., Dawson, E., & Huskey, A. (2021). Polysubstance Use Among Patients Treated With Buprenorphine From a National Urine Drug Test Database. *JAMA network open*, 4(9), e2123019. <https://doi.org/10.1001/jamanetworkopen.2021.23019>
35. Ford, B. R., Bart, G., Grahon, B., Shearer, R. D., & Winkelman, T. N. A. (2021). Associations Between Polysubstance Use Patterns and Receipt of Medications for Opioid Use Disorder Among Adults in Treatment for Opioid Use Disorder. *Journal of addiction medicine*, 15(2), 159–162. <https://doi.org/10.1097/ADM.0000000000000726>
36. Lin, L. A., Bohnert, A. S. B., Blow, F. C., Gordon, A. J., Ignacio, R. V., Kim, H. M., & Ilgen, M. A. (2021). Polysubstance use and association with opioid use disorder treatment in the US Veterans Health Administration. *Addiction (Abingdon, England)*, 116(1), 96–104. <https://doi.org/10.1111/add.15116>
37. Xu, K. Y., Mintz, C. M., Presnall, N., Bierut, L. J., & Grucza, R. A. (2022). Comparative Effectiveness Associated With Buprenorphine and Naltrexone in Opioid Use Disorder and Cooccurring Polysubstance Use. *JAMA network open*, 5(5), e2211363. <https://doi.org/10.1001/jamanetworkopen.2022.11363>
38. American Society of Addiction Medicine. (2020). *The ASAM National Practice Guideline for the Treatment of Opioid Use Disorder: 2020 Focused Update*. <https://www.asam.org/quality-care/clinical-guidelines/national-practice-guideline>
39. Martin, S. A., Chiodo, L. M., Bosse, J. D., & Wilson, A. (2018). The Next Stage of Buprenorphine Care for Opioid Use Disorder. *Annals of internal medicine*, 169(9), 628–635. <https://doi.org/10.7326/M18-1652>
40. U.S Food & Drug Administration. (2017, September 26). *FDA Drug Safety Communication: FDA urges caution about withholding opioid addiction medications from patients taking benzodiazepines or CNS depressants: careful medication management can reduce risks*. <https://www.fda.gov/drugs/drug-safety-and-availability/fda-drug-safety-communication-fda-urges-caution-about-withholding-opioid-addiction-medications>
41. Samuels, E. A., D'Onofrio, G., Huntley, K., Levin, S., Schuur, J. D., Bart, G., Hawk, K., Tai, B., Campbell, C. I., & Venkatesh, A. K. (2019). A Quality Framework for Emergency Department Treatment of Opioid Use Disorder. *Annals of emergency medicine*, 73(3), 237–247. <https://doi.org/10.1016/j.annemergmed.2018.08.439>